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HIS GLOBAL CONCLAVE- EDITION 3.0

"Where Young Minds Meet to Debate, Deliberate and Lead."

WORLD HEALTH ORGANIZATION
BACKGROUND GUIDE

SDG 3 – Good Health and Well-Being
Agenda: The Outbreak of The Covid-19 Virus

Delegates will examine the global health crisis, its international impact and the role of the global health community in managing public health emergencies.

Dear Delegates,

We extend a warm welcome to each of you at HIS Global Conclave Edition 3. As delegates of the World Health Organization (WHO), you will be stepping into the roles of decision-makers facing one of the defining public health emergencies of our time: the COVID-19 pandemic.

The outbreak demonstrated that infectious diseases do not respect borders. It challenged health systems, disrupted education and livelihoods, intensified inequalities, and tested the ability of countries to share information, resources, and scientific knowledge. It also showed the importance of health workers, researchers, public communication, and coordinated action.

Your committee will consider how the international community can respond to outbreaks quickly while protecting public trust, human rights, and access to essential healthcare. Delegates are encouraged to examine both the successes and the shortcomings of the global response, recognizing that countries faced different capacities, resources, and circumstances.

The WHO works with Member States and partners to provide scientific guidance, coordinate international health action, and support stronger health systems. However, effective action also depends on national governments, regional bodies, medical professionals, communities, and individuals. Your task is to build proposals that respect these different responsibilities while promoting solidarity.

We encourage you to research your assigned country's healthcare system, pandemic experience, priorities, and policy positions. Listen carefully, negotiate constructively, and aim for resolutions that are practical, inclusive, and grounded in evidence.

May this committee be a space for thoughtful debate and meaningful collaboration. The decisions discussed here should reflect a central principle: preparedness and healthcare access must be strengthened for everyone, not only for those with the greatest resources.

Sincerely,

Chairperson & Vice Chairperson

WHO

History of the Committee

The World Health Organization (WHO)

The World Health Organization is the United Nations' specialized agency for international public health. Its Constitution was adopted in 1946 and entered into force on 7 April 1948, a date now observed annually as World Health Day. WHO's founding vision recognizes health as a fundamental part of human well-being and international cooperation.

WHO works with countries and partners to promote health, keep the world safe, and serve vulnerable populations. Its work includes setting health standards and guidance, monitoring health trends, supporting disease prevention and control, strengthening health systems, coordinating responses to international health emergencies, and encouraging research and evidence-based policy.

WHO is governed by the World Health Assembly, where Member States determine policies, appoint the Director-General, and approve the Organization's programmes and budget. The Executive Board helps implement the Assembly's decisions and advises it. WHO also operates through regional offices and country offices, supporting cooperation across different health contexts.

WHO's role has limits. It provides leadership, technical guidance, coordination, and support, but it does not replace national governments or operate as a world health police force. Countries remain responsible for implementing public health measures within their jurisdictions. International cooperation is therefore essential: information must be shared promptly, and national actions need to be coordinated when risks cross borders.

Why WHO matters during an outbreak

- It brings countries and technical experts together to assess emerging health threats.
- It publishes guidance on surveillance, testing, infection prevention, clinical care, and public communication.
- It supports countries with weaker health systems and helps coordinate supplies, expertise, and partnerships.
- It promotes international standards and encourages transparent reporting of public health risks.
- It helps identify gaps in preparedness and supports lessons learned after emergencies.

The COVID-19 crisis highlighted both the value of a shared global health institution and the need to improve international preparedness, cooperation, and trust.

Introduction to the Agenda

Agenda 1: The Outbreak of the COVID-19 Virus

COVID-19 is an infectious disease caused by the coronavirus SARS-CoV-2. A cluster of pneumonia cases in Wuhan, China, was reported to the WHO at the end of December 2019. As the virus spread internationally, countries faced urgent decisions about surveillance, testing, healthcare capacity, public communication, travel, and the protection of vulnerable communities.

On 30 January 2020, WHO declared the outbreak a Public Health Emergency of International Concern (PHEIC) under the International Health Regulations. On 11

March 2020, WHO assessed that COVID-19 could be characterized as a pandemic, reflecting the extent of global spread. These were different milestones: a PHEIC is a formal international alert under the Regulations, while “pandemic” describes widespread transmission across countries and regions.

The crisis affected more than hospitals. Border restrictions and lockdowns disrupted trade, employment, tourism, schooling, and family life. Health services for other conditions were interrupted in many places, while misinformation and unequal access to tests, treatments, protective equipment, and vaccines complicated the response. Health workers faced extraordinary demands, and communities experienced grief, isolation, financial strain, and learning loss.

The agenda asks delegates to examine how the international community can improve outbreak detection, information-sharing, emergency coordination, equitable distribution of medical countermeasures, and continuity of essential health services. Delegates should also consider how to balance urgent public health measures with education, livelihoods, human rights, privacy, and public confidence.

SDG 3 – Good Health and Well-being

Sustainable Development Goal 3 aims to ensure healthy lives and promote well-being for all at all ages. COVID-19 threatened progress on universal health coverage, the prevention and treatment of communicable diseases, access to essential medicines and vaccines, and the resilience of health systems. The pandemic demonstrated that achieving SDG 3 requires investment before emergencies occur, not only after they begin.

A successful response should protect those at greatest risk, support health workers, maintain essential services, make decisions transparent, and ensure that low-resource settings are not left behind.

The Outbreak: Key Milestones and Global Impact

Selected international milestones

- **31 December 2019:** WHO was informed of a cluster of pneumonia cases in Wuhan, China.

- **30 January 2020:** WHO declared the outbreak a Public Health Emergency of International Concern (PHEIC).
- **11 February 2020:** WHO announced the disease name COVID-19; the virus was later named SARS-CoV-2.
- **11 March 2020:** WHO assessed that COVID-19 could be characterized as a pandemic.
- **2020 –2021:** Countries expanded testing, public health measures, clinical care, and vaccination programmes, with different levels of access and capacity.
- **5 May 2023:** WHO determined that COVID-19 no longer constituted a PHEIC. WHO emphasized that this did not mean COVID-19 had ceased to be a global health threat.

Health and social consequences

COVID-19 caused substantial illness and loss of life and placed severe pressure on hospitals, laboratories, supply chains, and healthcare workers. The pandemic also exposed gaps in access to primary care, oxygen, personal protective equipment, diagnostics, treatments, and vaccines. Countries with limited resources often faced additional barriers in obtaining supplies and delivering services at scale.

Economic disruption affected workers and businesses, especially those dependent on in-person activity. School closures interrupted learning and widened existing inequalities. Restrictions reduced some forms of social contact but also contributed to isolation and stress. In many settings, routine services such as immunization, maternal care, chronic disease treatment, and screening were delayed or disrupted.

The response also produced major scientific advances. Researchers rapidly shared genomic information, developed diagnostic tools, evaluated treatments, and created vaccines in record time. These achievements illustrated the power of international scientific collaboration, while uneven access to those advances highlighted the importance of fair distribution and strong delivery systems.

Key International Frameworks and Achievements

The International Health Regulations (2005)

The International Health Regulations (IHR) are a legally binding framework that helps countries prevent, prepare for, and respond to public health risks that may cross borders. They set out responsibilities for surveillance, assessment, notification, and response, while seeking to avoid unnecessary interference with international traffic and trade. COVID-19 exposed implementation gaps and prompted renewed attention to preparedness, transparency, and coordination.

In 2024, WHO Member States adopted amendments to the IHR. These amendments entered into force for most States Parties in September 2025 and included a new "pandemic emergency" alert level, National IHR Authorities, and stronger provisions relating to coordination, equity, and financing.

The WHO Pandemic Agreement

On 20 May 2025, the World Health Assembly adopted the WHO Pandemic Agreement. It was developed in response to gaps and inequities revealed by COVID-19, with the aim of strengthening pandemic prevention, preparedness, and response. The agreement seeks to improve international cooperation, make health systems more resilient, and support more equitable access to pandemic-related health products.

The 2030 Agenda and SDG 3

Adopted by UN Member States in 2015, the 2030 Agenda includes 17 Sustainable Development Goals. SDG 3 covers universal health coverage, reducing deaths from communicable and non-communicable diseases, strengthening prevention and treatment, and improving access to medicines and vaccines. Pandemic preparedness is closely connected to these targets because outbreaks can reverse health gains and widen inequalities.

International scientific collaboration

WHO coordinated guidance and research priorities, supported international information-sharing, and worked with governments and partners during the emergency. Research networks, public health agencies, universities, manufacturers, and healthcare workers contributed to advances in diagnostics,

treatments, vaccines, and clinical knowledge. These efforts offer a basis for stronger systems, but they also raise questions about affordability, intellectual property, technology transfer, and fair allocation.

A Call to Delegates

As delegates of the World Health Organization, you are entrusted with addressing a challenge that revealed how closely the health of one country is connected to the health of the world. COVID-19 showed that no country can prepare for every emergency in isolation. Timely information, scientific cooperation, resilient health systems, and equitable access to healthcare are essential to reducing the impact of future outbreaks.

Your task is not simply to review what went wrong, but to propose what the international community can do better. Consider the needs of countries with different capacities, the protection of vulnerable communities, and the importance of trust between governments and the public. Strong resolutions should identify who will act, how proposals will be funded, and how progress will be measured.

Aim for resolutions that are specific, cooperative, evidence-based, financially realistic, and measurable. Avoid proposals that assign all responsibility to WHO alone; implementation requires Member States and a wide network of partners.

Let your debate reflect the purpose of SDG 3: ensuring healthy lives and promoting well-being for all. Through diplomacy, evidence, and shared responsibility, delegates can help shape a more prepared and equitable global health system.

Best of luck, delegates!

Sincerely,

Chairperson & Vice Chairperson

WHO